

01

Foundations & Systematic Assessment

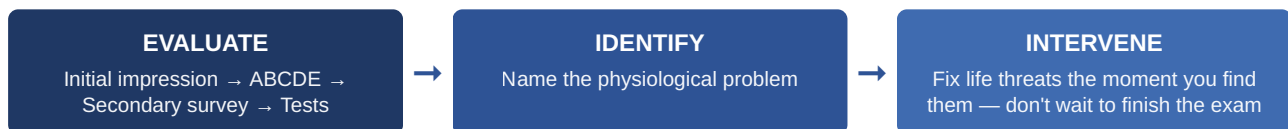
Master systematic assessment (PAT → ABCDE → Secondary Survey → Early Escalation) and recognize the sick child within the first 5 minutes.

1. Foundations & Systematic Assessment

Learning Objectives

- Apply the Evaluate → Identify → Intervene sequence to every child on first contact.
- Form an accurate Initial Impression using the Paediatric Assessment Triangle (PAT) before touching the child.
- Complete a structured ABCDE primary assessment and recognise the five-point breathing and circulation checks.
- Take a focused SAMPLE history and apply the DIRECT approach to organise a bedside handover.
- Assign an accurate triage acuity level and know when to escalate immediately.

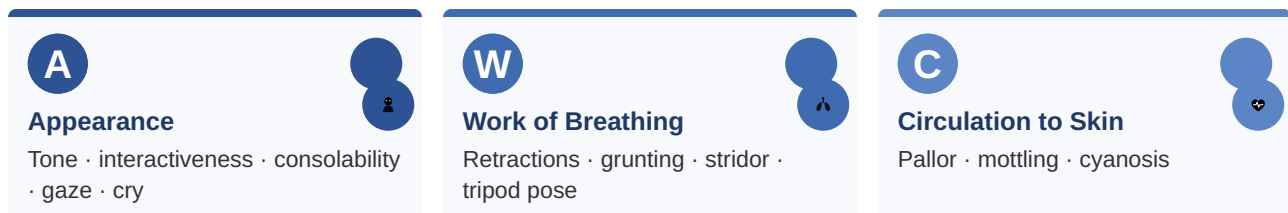
1. The Loop: Evaluate → Identify → Intervene



WHY THIS MATTERS

Out-of-hospital arrest survival ≈10%; in-hospital ≈30%. Respiratory & circulatory failure is usually preventable if caught in the first 5 minutes.

2. Initial Impression — PAT (from the doorway, hands off)



2 abnormal limbs → sick child, urgent primary assessment. **3 abnormal** → impending cardiopulmonary failure.

3. Primary Assessment — ABCDE

A
Airway

Talking/crying normally? No gurgle/stridor? Check head position, suction needs.

B
Breathing

5-point check → rate, air entry, sounds, effort, SpO₂ (see box below)

C
Circulation

5-point check → pulse, volume, CRT, temp/colour, BP

D
Disability

AVPU · pupils · glucose in every drowsy child

E
Exposure

Fully undress — look for bruising/petechiae; prevent hypothermia


B — Breathing, 5-Point Check

- 1 Rate for age — >60 or <10/min = danger
- 2 Equal air entry
- 3 Sounds: stridor / wheeze / grunt
- 4 Effort: flaring, recession, see-saw
- 5 SpO₂ <94% abnormal


C — Circulation, 5-Point Check

- 1 Pulse rate/rhythm — <60/min ominous
- 2 Pulse volume — central-peripheral gap = earliest shock sign
- 3 CRT — press 5s, >2s prolonged
- 4 Temp/colour/mottling
- 5 BP — remember **60–70–80–90** (min systolic by age)


D & E IN ONE LINE

AVPU: Alert → Voice → Pain → Unresponsive — P/U mandates emergency referral. Check glucose (<60 mg/dL = treat now) and fully expose while preventing hypothermia.

4. Secondary Assessment — SAMPLE

S
Signs & Symptoms

Of the presentation

A
Allergies
M
Medications

Given so far

P
Past History

Medical background

L
Last Meal

Timing

E
Events

Leading up to presentation

Follow with a focused head-to-toe exam. Basic work-up: glucose, CBC, CRP, electrolytes, LFT/RFT, gas + lactate; CXR if respiratory; ECG/ECHO if cardiac; USG if abdo/renal concern.

5. Triage — Assigning Acuity

Level	Category	Example	Immediate action
1	Resuscitation	Arrest, active seizure, hypotensive shock	CPR, intubation
2	Emergent	Compensated shock, distress	O ₂ , NS bolus
3	Urgent	Dehydration, post-ictal, pain	Monitor, treat

4	Less urgent	Stable, minor injury	Can wait
5	Non-urgent	Minor illness	Routine

1 Visual assessment
First glance from across the room



2 Chief complaint + SAMPLE
Focused history



3 Focused ABCD
Quick targeted exam



4 Form a hypothesis
What's most likely going on?



5 Assign acuity
Level 1–5



6 Reassess
Re-triage if condition changes while waiting

6. DIRECT — Bedside Handover



D-I-R

Presentation given · Interventions already done ·
Response to treatment so far



E-C-T

Evolving concerns · Care plan · Tasks pending for next
shift

Use this structure at every shift change and escalation call so nothing is lost in translation.

7. Ready the Environment



START-OF-SHIFT CHECKLIST

O₂ supply · suction · infusion lines · ventilator/BiPAP/HFNC · monitor alarms · crash cart layout
by heart (airway / breathing / circulation-access / drugs / defib-ECG / misc) · Broselow tape
ready for unknown-weight children.

**NURSE SELF-CARE**

A calm, rested, focused nurse is more effective than one rushing in drained. Your own wellbeing is foundational to the child's safety.

Compiled & adapted from

- PEERS Manual (2025) — Section 1: Recognition of a Sick Child; Section 2: Principles of Triage (pp. 4–14, 12–14, Appendix vitals tables).
- BPCCN Guide — Daily Routine of PICU Nurses; Identification of a Sick Child; Triage and its Levels in Paediatric Emergency (pp. 2–22).

Page-level citations to be added in a subsequent pass. Content has been edited and condensed for print/online study use; refer to the full source manuals for exhaustive detail.